One Community
HOME HEALTH
Care That Feels Like Home

Employee Handbook

and Policy Manual

Policies, procedures, and resources for every member of our team

Keep this handbook for reference.
Read it during orientation, keep it where you can find it, and ask your supervisor any time you have a question. You will sign a receipt confirming that you received it.

JACOP Healthcare Services, Inc., doing business as One Community Home Health
3560 Quannah Drive, Grand Prairie, Texas 75052 • 972-325-1598

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Quick Reference: Who to Call

Situation	Call	Number
Life-threatening emergency (client or you)	911 first, then the office	911
Questions, schedule changes, call-offs, incidents (any time)	One Community Home Health office / on-call nurse	972-325-1598
Suspected abuse, neglect, or exploitation of a client	Texas Abuse Hotline (DFPS)	1-800-252-5400 • txabusehotline.org
Complaint about an agency, or ANE by agency staff	HHSC Complaint and Incident Intake	1-800-458-9858
Possible poisoning	Poison Control	1-800-222-1222
Privacy or security concern (lost phone, wrong fax, misdirected record)	Privacy Officer	972-325-1598 • privacy@onechh.com
Compliance concern (fraud, billing, kickbacks)	Compliance Officer (anonymous reports accepted)	972-325-1598
Emotional crisis (you or anyone)	988 Suicide & Crisis Lifeline	Call or text 988

The five rules that matter most

- Client safety comes first. In an emergency, call 911, then the office.
- Report suspected abuse, neglect, or exploitation right away — it is the law.
- Clock in and out of every visit, and document only the care you actually gave.
- Keep client information private. Never post, text, or talk about clients outside the care team.
- Stay within your role. If a task is not on the care plan or you are not trained for it, call your supervisor.

PART 1

About Us



1.1 Welcome

Welcome to One Community Home Health. Our team cares for older adults, people with disabilities, veterans, and children in the place they most want to be: home. The work you do lets families stay together, keeps people out of the hospital, and protects their dignity. We are glad you are here.

This handbook explains what you can expect from us and what we expect from you. It brings together, in one place, the policies you sign at hire — confidentiality, the drug-free workplace, harassment, abuse reporting, electronic visit verification, and the rest — so that you always know where to find the full rule behind each signature.



1.2 Our Purpose and Values

Our purpose is to provide safe, skilled, and compassionate care at home, delivered by people our clients trust. We do that by living these values:

- **Dignity.** Every client is treated with respect for his or her privacy, culture, faith, and choices.
- **Safety.** We follow the care plan, use standard precautions, and speak up when something is wrong.
- **Integrity.** We document truthfully, bill honestly, and keep our promises to clients and to each other.
- **Teamwork.** Nurses, therapists, aides, attendants, and office staff work as one team around each client.
- **Community.** We serve our neighbors across North Texas and take pride in being local.



1.3 Services, Service Area, and Payers

JACOP Healthcare Services, Inc., doing business as One Community Home Health, is licensed by the Texas Health and Human Services Commission (HHSC) as a home and community support services agency (HCSSA). We provide skilled home health services under physician or practitioner orders and personal assistance services under an individualized service plan.

Skilled home health	Personal assistance and attendant services
Skilled nursing (RN and LVN)	Bathing, grooming, dressing, toileting
Physical, occupational, and speech-language therapy	Ambulation, transfers, positioning
Medical social work	Meal preparation, light housekeeping, laundry

Skilled home health	Personal assistance and attendant services
Home health aide services	Escort, shopping, companionship, caregiver respite

We serve clients in Collin, Dallas, Denton, Ellis, Johnson, Kaufman, Rockwall, and Tarrant Counties. Payers include Medicare, Texas Medicaid and Medicaid managed care organizations, the U.S. Department of Veterans Affairs (Community Care), and private pay. Each payer has its own documentation and authorization rules; your supervisor will tell you which apply to each client.



1.4 Who Does What

Role	Responsibilities
Administrator	Overall operation, licensure, personnel, complaints, compliance with HHSC rules
Alternate Administrator	Acts for the Administrator when absent
Supervising Nurse / Director of Nursing	Clinical services, care plans, supervision of nurses, aides, and attendants
Alternate Supervising Nurse	Acts for the Supervising Nurse when absent
Privacy Officer	HIPAA and Texas privacy law, breach response, privacy training
Compliance Officer	Fraud, waste, and abuse prevention; exclusion screening; compliance reports
Scheduler / Office Coordinator	Schedules, call-offs, EVV visit maintenance, timesheets

The names and phone numbers of the people in these roles are on the Staff Contact Sheet (Form E25). You receive a copy at orientation, a copy is posted in the office, and the office gives you an updated copy whenever a role changes hands.

PART 2

Employment Basics



2.1 About This Handbook

This handbook summarizes the Agency's personnel policies and the rules that apply to client care. It is not a contract of employment and does not promise employment for any length of time. Detailed clinical and administrative procedures are kept in the Agency's Policy and Procedure Manual at the office; your supervisor will show you where it is during orientation. If this handbook and a law, regulation, or written payer requirement ever conflict, the law, regulation, or payer requirement controls.

The Agency may change, add, or remove policies at any time. You will be told about material changes in writing, and you may be asked to sign an updated acknowledgment.



2.2 At-Will Employment

Employment with One Community Home Health is at will. You may resign at any time, and the Agency may end the employment relationship at any time, with or without cause or notice, for any reason that is not unlawful. Only the Administrator may change the at-will relationship, and only in a written agreement signed by the Administrator and the employee.



2.3 Equal Employment Opportunity and Reasonable Accommodation

The Agency provides equal employment opportunity to all employees and applicants without regard to race, color, religion, sex (including pregnancy, sexual orientation, and gender identity), national origin, age (40 and over), disability, genetic information, military or veteran status, or any other characteristic protected by federal, state, or local law. This applies to hiring, pay, assignments, training, promotion, discipline, and termination.

The Agency will make reasonable accommodations for qualified individuals with disabilities, for known limitations related to pregnancy, childbirth, or related conditions, and for sincerely held religious beliefs, unless the accommodation would cause undue hardship. To request an accommodation, talk to the Administrator. You do not need to use any special words. The Agency will work with you to find an effective accommodation and will keep medical information confidential.

Assignments are made based on client needs, the care plan, and staff qualifications. A client's request for a caregiver of a particular race, color, religion, or national origin will not be honored. A client's request based on sex for intimate personal care will be considered case by case, consistent with law.

2.4 Harassment, Discrimination, and Retaliation

Everyone deserves a workplace free of harassment. The Agency prohibits harassment based on any protected characteristic, including sexual harassment, by anyone — supervisors, co-workers, clients, family members, vendors, or visitors.

Sexual harassment includes unwelcome sexual advances, requests for sexual favors, and other verbal, visual, or physical conduct of a sexual nature when submission is made a condition of employment, is used as a basis for employment decisions, or creates an intimidating, hostile, or offensive work environment. Examples include sexual jokes, comments about a person's body, sexual pictures or messages, unwanted touching, and repeated requests for dates after being told no.

How to report. Report harassment you experience or witness to your supervisor, the Administrator, or any member of management — in person, by phone at 972-325-1598, or in writing. If the person you would normally report to is involved, go to someone else. If a client or family member harasses you, tell the office right away; your safety comes first, and you may leave the home if you feel unsafe after calling the office.

What happens next. The Agency will investigate promptly and as confidentially as possible, and will take corrective action, up to and including termination, when it finds a violation. The Agency will tell you when the investigation is complete.

No retaliation. The Agency will not retaliate, and will not allow anyone to retaliate, against a person who makes a good-faith report or takes part in an investigation. You may also contact the U.S. Equal Employment Opportunity Commission (eeoc.gov, 1-800-669-4000) or the Texas Workforce Commission Civil Rights Division (twc.texas.gov).

You signed for this policy on the Employee Acknowledgment of Agency Policies (Form E05).

2.5 Employment Categories and Introductory Period

Category	Meaning
Full-time	Regularly scheduled 30 or more hours per week
Part-time	Regularly scheduled fewer than 30 hours per week
PRN / as needed	No regular schedule; works when assigned and available
Per-visit (clinicians)	Paid per completed, documented visit as stated in the offer letter
Nonexempt	Entitled to minimum wage and overtime under the Fair Labor Standards Act (most field staff and office staff)
Exempt	Salaried professional or management positions that meet federal exemption tests

Your first 90 days are an introductory period. Your supervisor will observe your work, give you feedback, and complete an evaluation at the end of the period. Completing the introductory period does not change the at-will relationship.

2.6 Screening Before Hire and While Employed

Texas law and HHSC rules require the Agency to screen every person who will have contact with clients. Before you may work, and as long as you work here, the Agency will:

- **Criminal history:** run a Texas Department of Public Safety criminal history check. A conviction for an offense listed in Texas Health and Safety Code § 250.006 bars employment in a position with direct client contact. Other convictions are reviewed individually (Form E20).
- **Registries:** search the HHSC Nurse Aide Registry and Employee Misconduct Registry through the HHSC Texas Unified Licensure Information Portal (TULIP) before hire and at least every 12 months. A person listed as unemployable may not be employed.
- **Exclusion lists:** check the HHS Office of Inspector General List of Excluded Individuals/Entities and the Texas HHSC-OIG exclusion list before hire and monthly. An excluded person may not work on any service paid by Medicare or Medicaid.
- **Licenses and certifications:** verify licenses with the issuing board before hire and on renewal.
- **Work authorization:** complete Form I-9 on your first day (Section 1) and within three business days (Section 2).

You must tell us within 24 hours if you are:

- arrested, charged, or convicted of any offense other than a minor traffic violation;
- the subject of an investigation, complaint, or disciplinary action by a licensing board, DFPS, or HHSC;
- excluded from Medicare or Medicaid; or
- if your license or certification expires, lapses, or is restricted. Failing to report is grounds for termination.

2.7 Personnel Files and Where Your Documents Live

Every document you sign at hire and during employment is filed in a specific place. Appendix A lists each form, where it is filed, and how long it is kept. In summary:

File	What is in it	Who may see it
Personnel file (Sections 1–5)	Application, screening results, signed policies, job description, training, competency, evaluations, discipline	Administrator, supervisors as needed, HHSC surveyors, and you on request
Confidential medical file	TB screening, Hepatitis B and influenza forms, accommodation and medical information, work injury records	Administrator and Privacy Officer only
Form I-9 file	Form I-9 and copies of identity documents	Administrator; government inspectors

File	What is in it	Who may see it
Criminal history file	DPS CCH Verification Form; review memos. Criminal history records are not stored after the decision is made	Administrator only

Personnel files are kept in a locked cabinet at the office and in the Agency's secure electronic HR system. You may review your own personnel file by appointment with the Administrator. Tell the office within 7 days if your name, address, phone number, email, emergency contact, or tax withholding changes.



2.8 Performance Evaluations

Your supervisor will evaluate your performance at the end of your introductory period and at least once a year. Evaluations for home health aides and attendants include on-site observation of care in the client's home. You will receive a copy and may add written comments.



2.9 Conflicts of Interest, Outside Work, and Relationships

You may hold another job as long as it does not interfere with your schedule, create a conflict of interest, or involve working privately for an Agency client. You may not accept private employment from a current client or from a client's family for 12 months after you last served that client without the Administrator's written approval. Disclose any relationship with a referral source, vendor, or competitor on the Confidentiality and Conflict of Interest Disclosure (Form E15).

Romantic or sexual relationships with clients or clients' family members are prohibited. A supervisor may not date or supervise a family member or a person he or she is dating; tell the Administrator so assignments can be adjusted.

PART 3

Pay, Time, and Leave



3.1 Workweek and Payday

The workweek begins Sunday at 12:00 a.m. and ends seven days later. Employees are paid every other Friday (biweekly) by direct deposit or pay card. Texas law requires nonexempt employees to be paid at least twice a month.



3.2 Electronic Visit Verification and Timekeeping

HHSC requires electronic visit verification (EVV) for Medicaid personal attendant and home health services. The Agency uses HHAeXchange. You must:

- clock in when you begin and clock out when you finish every visit, using the HHAeXchange mobile app or the client's home phone (landline) as trained;
- never clock in or out for another person, or let anyone clock for you;
- call the office the same day if you forget to clock, the app fails, or the visit does not match the schedule, so the office can complete visit maintenance with the correct reason code;
- complete the visit note or task list before you leave the home (Medicare visits are documented in the clinical record; personal care tasks are documented on the service record).

Falsifying time or visit records is fraud. It is grounds for immediate termination and may be reported to HHSC, the HHSC Office of Inspector General, and law enforcement.

You signed for this policy on Form E05 (Electronic Visit Verification and Documentation).



3.3 Overtime and Travel Time

Overtime. Nonexempt employees, including home care workers employed by the Agency, are paid one and one-half times their regular rate for all hours worked over 40 in a workweek, as required by the Fair Labor Standards Act. Overtime must be approved in advance by the scheduler or your supervisor, but all hours actually worked will be paid. (The U.S. Department of Labor has proposed changing how overtime applies to agency-employed companions; the Agency will follow the law in effect and will notify you of any change.)

Travel time. Time spent traveling from one client to another during the workday is work time and is paid. Your normal commute from home to your first client and from your last client back home is not work time.

Training and meetings. Required orientation, in-services, and staff meetings are paid work time.

3.4 Breaks

Texas does not require meal or rest breaks for adults, but the Agency encourages them. Short rest breaks (20 minutes or less) are paid. A meal break of 30 minutes or more is unpaid only if you are completely relieved of duty. Never leave a client who cannot safely be left alone in order to take a break; call the office if you need relief.

3.5 Pay Questions and Final Pay

Check every pay stub. If you believe your pay is wrong, tell the office in writing right away; errors are corrected no later than the next regular payday. The Agency makes only deductions required by law or authorized by you in writing.

Under the Texas Payday Law, if you are discharged, your final pay is due within six calendar days. If you resign, your final pay is due on the next regular payday. Return all Agency property — ID badge, supplies, keys, and client records — when you leave.



3.6 Paid Time Off, Holidays, and Benefits

The Agency does not currently provide paid time off or paid holidays. If the Agency adds paid time off, holiday pay, or other benefits in the future, employees will be told in writing what the benefits are and who is eligible. Because clients need care every day, field staff may be scheduled on holidays.

3.7 Leaves of Absence

Leave	What you need to know
Personal or medical leave	Request in writing at least two weeks ahead when possible. Approval depends on staffing and client needs. Leave is unpaid.
Family and Medical Leave Act (FMLA)	Applies when the Agency employs 50 or more employees. If it applies to you, the Agency will give you the required FMLA notices.
Pregnancy	Reasonable accommodations are available for pregnancy, childbirth, and related conditions. Talk to the Administrator.
Jury duty	Texas law protects your job when you serve on a jury. Give the office a copy of your summons.

Voting	You may take time off to vote on election day unless you have two consecutive hours outside of work while the polls are open. You will not be penalized for doing so.
Military service	Your job rights are protected by the Uniformed Services Employment and Reemployment Rights Act (USERRA) and Texas law.



3.8 Work Injuries and Workers' Compensation



If you are hurt at work

- Get emergency care if needed (call 911 or go to the nearest emergency room).
- Make sure the client is safe, then call the office at 972-325-1598 immediately — and no later than 24 hours.
- Complete the Employee Incident / Injury Report with your supervisor.
- Needlesticks and blood exposures are reported immediately so you can receive prompt medical evaluation (see 5.8).

One Community Home Health does not carry workers' compensation insurance (the Agency is a "nonsubscriber" under the Texas Workers' Compensation Act). As an employee of a non-covered employer, you are not eligible to receive workers' compensation benefits under the Texas Workers' Compensation Act. You receive written notice of this at hire (Form E24), and the official Texas Notice 5 is posted at the office in English and Spanish.

Reporting every injury promptly still matters: it gets you care, protects clients, and lets the Agency file the injury reports Texas requires of non-covered employers. The Agency does not offer an occupational injury benefit plan, so it does not pay medical bills or lost wages for work injuries through a benefit plan. Your own health insurance may cover treatment; check with your insurer.

For questions about your rights, call the Texas Department of Insurance, Division of Workers' Compensation, at 1-800-252-7031 or visit tdi.texas.gov/wc. To report unsafe working conditions, call the 24-hour safety violations hotline at 1-800-452-9595; the law prohibits retaliation against employees who report in good faith.

You signed for this policy on Form E05 (Workplace Injury).

PART 4

Standards of Conduct



4.1 Professional Boundaries in the Client's Home

You are a guest in every client's home, and our clients are often vulnerable. Clear boundaries protect them and protect you. While working for the Agency you must not:

- accept money, tips, gifts, loans, or property from a client or family, or be named in a client's will; politely decline and tell the office about any offer;
- borrow from or lend to a client, sell anything to a client, or ask a client to buy anything or donate to any cause;
- use the client's phone, car, food, or belongings for personal purposes, or handle the client's money, bank cards, or checks unless the care plan specifically allows it and it is documented;
- bring children, friends, pets, or anyone else to a client's home, or conduct personal business during a visit;
- smoke or vape in a client's home or vehicle, or in front of a client;
- sleep on duty, watch television or use your phone for personal reasons while providing care;
- discuss your personal problems, religion, or politics in a way that pressures the client, or give personal advice outside your role;
- give clients your personal phone number or social media contact, or connect with clients or families on social media;
- take photographs or video of a client or the home, except as directed by your supervisor for the clinical record using an approved device.



4.2 Confidentiality and Privacy (HIPAA and Texas Law)

Client health information is protected by HIPAA, the HITECH Act, and the Texas Medical Records Privacy Act (Health and Safety Code Chapter 181). Texas law requires the Agency to train you on privacy within 90 days of hire and to keep your signed training statement for six years (Form E19).

- **Minimum necessary.** Look at, use, and share only the information you need to do your job.
- **No public talk.** Do not discuss clients in public places, with your family, or with other clients.
- **Social media.** Never post anything about a client — not a name, photo, location, story, or "anonymous" description — on any platform, even in private groups.
- **Texts and email.** Send client information only through Agency-approved, secure channels. Do not text client names or diagnoses from a personal messaging app.
- **Devices.** Lock your phone or tablet with a passcode. Never share passwords (Form E16). Report a lost or stolen device to the office immediately.
- **Paper.** Keep paperwork in your bag, never visible in your car, and return it to the office on time.

Report any privacy problem immediately — a lost device, a fax or email sent to the wrong place, a record left behind, or a conversation overheard. Call the Privacy Officer at 972-325-1598 or email privacy@onechh.com. Reporting quickly is what limits the harm.

You signed for this policy on Forms E05, E15, E16, and E19. Obligations continue after your employment ends.



4.3 Drug-Free and Alcohol-Free Workplace

You may not make, sell, distribute, possess, or use illegal drugs, or use or possess alcohol, while on duty, on Agency premises, in a client's home, or in any vehicle used for Agency business. You must report to work fit for duty. Prescription medication is allowed if it does not impair your ability to work safely; tell your supervisor before working if a medication may cause impairment.

The Agency may require drug and alcohol testing before employment (after a conditional offer), on reasonable suspicion, after an accident or client incident, before returning to work after a violation, and on a random basis, as permitted by law. Refusal to test or tampering with a specimen is treated as a positive result. Violations lead to discipline up to and including termination and may be reported to a licensing board. Clients receive a copy of this policy (Form A19).

Full policy: Drug-Free Workplace Policy (Form E06), signed at hire.



4.4 Appearance and Identification

- Wear clean, wrinkle-free solid-color scrubs or the uniform your supervisor assigns, with closed-toe, non-slip shoes.
- Wear your Agency photo ID badge, visible, at every visit.
- Keep fingernails short and clean; no artificial nails or nail extensions for staff who provide hands-on care (infection control).
- Keep jewelry minimal and hair secured away from the face; avoid strong perfume or cologne, which can trouble clients with breathing problems.



4.5 Attendance, Call-Offs, and Schedule Changes

Clients depend on you. A missed visit can mean a missed medication, a fall, or a person left without food or care.

- **Calling off.** Call the office at 972-325-1598 at least 4 hours before a scheduled visit (or as soon as possible in an emergency). Speak to a person — do not leave a voicemail, text, or ask a co-worker to tell us.
- **Running late.** Call the office before your visit start time.
- **Shift relief.** If the next caregiver does not arrive, stay with a client who cannot be left alone and call the office. We will arrange relief as quickly as possible.

- **Changes requested by the client.** Do not agree to change days, hours, or tasks directly with a client. Refer the request to the office; changes must match the plan of care, authorization, and EVV schedule.
- **No-call/no-show.** Missing a scheduled visit without calling is serious misconduct. Two no-call/no-shows in 12 months, or missing three scheduled workdays in a row without notice, will be treated as a voluntary resignation.



4.6 Driving and Phone Safety

If you drive as part of your job, you must keep a valid driver's license and the auto insurance Texas requires, and give the office a copy when either changes. Texas law prohibits reading, writing, or sending electronic messages while driving; the Agency also asks you not to make calls while driving. Pull over safely to call the office. You may transport a client only if it is part of the care plan and approved by your supervisor.



4.7 Workplace Safety, Violence, and Weapons

Threats, violence, intimidation, and bullying are not tolerated. Weapons are not allowed in clients' homes or at the office, except that Texas law permits a licensed employee to keep a lawfully owned firearm or ammunition locked in a private vehicle in the parking area. If you feel unsafe in a home — because of threats, weapons, illegal activity, an aggressive animal, or an impaired person — leave if you can do so safely, call 911 if needed, and then call the office.



4.8 Compliance: Fraud, Waste, Abuse, and Referrals

The Agency bills Medicare, Medicaid, and other government programs. Every employee must help keep our billing honest. Fraud includes billing for visits that did not happen, documenting care that was not given, signing another person's name, altering records, and clocking EVV for a visit you did not make.

Federal and state anti-kickback laws prohibit offering, paying, asking for, or accepting anything of value in exchange for client referrals. You may not offer gifts or incentives to clients, physicians, or facilities to win referrals, and you may not solicit Agency clients for another provider or for private work (Form E05, Non-Solicitation and Illegal Remuneration).



Speak up — you are protected

Report compliance concerns to your supervisor or the Compliance Officer at 972-325-1598. You may report anonymously. You may also report to the HHS Office of Inspector General (1-800-HHS-TIPS / 1-800-447-8477, tips.hhs.gov) or the Texas HHSC Office of Inspector General (1-800-436-6184). Federal and Texas law protect employees who report fraud in good faith from retaliation.



4.9 Progressive Discipline

The Agency usually corrects problems through progressive discipline: (1) verbal warning, documented; (2) written warning; (3) final written warning, which may include suspension; and (4) termination. Depending on how serious the conduct is, the Agency may skip steps or terminate employment immediately. Nothing in this section changes at-will employment.

Examples of conduct that may lead to immediate termination include:

Client safety and rights	Honesty and compliance	Workplace conduct
Abuse, neglect, or exploitation of a client	Falsifying time, EVV, or clinical records	Reporting to work impaired; drug or alcohol violation
Failing to report suspected abuse, neglect, or exploitation	Theft from a client or the Agency	Violence, threats, or weapons
Leaving a dependent client unattended without relief	Billing or kickback fraud; exclusion from Medicare or Medicaid	Harassment or discrimination
Serious privacy breach or social media posting about clients	Failing to report an arrest, conviction, or license action	No-call/no-show (see 4.5); job abandonment
Working outside your scope or the care plan in a way that endangers a client	Accepting money, gifts, or loans from clients	Insubordination; refusing a lawful assignment



4.10 Employee Concerns and Grievances

If you have a problem at work, talk with your supervisor first. If it is not resolved, or if your supervisor is involved, put your concern in writing to the Administrator. The Administrator will review it, may meet with you and others involved, and will respond in writing within 30 days. You will not be retaliated against for raising a concern in good faith.



4.11 Resigning

Please give at least two weeks' written notice so we can arrange care for your clients. Supervisors and office staff are asked to give three weeks. Return your ID badge, supplies, keys, and all client records, and complete any outstanding documentation before your last day. Employees who resign with proper notice and good standing may be considered for rehire.

PART 5

Client Care Standards

This part applies to everyone who visits clients: nurses, therapists, home health aides, attendants, and supervisors. Part 6 adds the rules for your specific role.



5.1 Client Rights

Every client receives a written copy of the Client Bill of Rights and Responsibilities (Form A13) and, if 60 or older, the Rights of the Elderly (Form A15). Know these rights and honor them at every visit. In particular, clients have the right to be treated with dignity and respect; to take part in planning their care; to refuse care after being told the consequences; to privacy and confidentiality; to voice complaints without retaliation; and to receive care without discrimination. If a client complains to you, listen, stay calm, and report the complaint to the office the same day.



5.2 Abuse, Neglect, and Exploitation — You Must Report

If you suspect abuse, neglect, or exploitation



- If the client is in immediate danger, call 911.
- **Report to DFPS immediately — no later than 24 hours: 1-800-252-5400 or txabusehotline.org.** For a child, report within 48 hours at the latest.
- Call the Administrator or Supervising Nurse right away, and document what you saw and the report number.
- Do not investigate on your own or confront the person you suspect.

Texas law requires anyone who has cause to believe that an elderly person, a person with a disability, or a child is being abused, neglected, or exploited to report it (Human Resources Code § 48.051; Family Code § 261.101). Failing to report is a crime. A person who reports in good faith is immune from liability. Signs to watch for include unexplained bruises or injuries, fearful behavior, poor hygiene, lack of food or medicine, unpaid bills, missing money or belongings, and a caregiver who will not let you speak with the client alone.

If an Agency employee is suspected, management will remove the employee from client contact while the Agency investigates, report to HHSC, and, if the allegation is confirmed, terminate employment and report to HHSC for the Employee Misconduct Registry and to any licensing board.

Full policy: Abuse, Neglect, and Exploitation Policy (Form A11). You signed for this policy on Form E05 and received the Employee Misconduct Registry notice (Form E18).



5.3 Following the Plan of Care and Staying Within Your Role

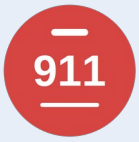
Each client has a written plan: a physician-ordered plan of care for skilled home health, an aide care plan for home health aides, or an individualized service plan for personal assistance services. Provide only the services on the plan, as often as the plan says, and only those you are qualified and trained to perform. If a client or family asks for something that is not on the plan, explain kindly that you must check with the office, and then call.

Unlicensed staff (home health aides and attendants) may not: administer medications (other than assisting with self-administration or performing tasks a registered nurse has delegated in writing under Texas Board of Nursing rules); give injections; perform sterile procedures; change a plan of care; take verbal orders from a physician; or give medical advice.



5.4 Documentation

- Document every visit the same day, before you leave the home whenever possible. Record what you actually did and observed, in your own words.
- Never document ahead of time, never document care someone else gave, and never sign another person's name or initials.
- Make corrections only as trained: in the electronic record, use the amendment function; on paper, draw a single line through the error, write "error," and initial and date it. Never use correction fluid or erase.
- If you remember something later, add a clearly labeled late entry with the current date and time.
- Your documentation must match your EVV clock-in and clock-out times.



5.5 Changes in Condition, Incidents, and Emergencies

Call the office before you leave the home — and immediately if it is urgent — when a client:

Report right away	Report the same day
Falls, is injured, or is found on the floor	Refuses care, medications, or a visit
Has chest pain, trouble breathing, stroke signs, or a major change in alertness	Has a new sore, rash, swelling, or skin breakdown
Is taken to the hospital or emergency room	Has changes in appetite, sleep, mood, or bowel or bladder habits
Dies or is found unresponsive	Has a home condition that is unsafe or unsanitary
Is not home or does not answer at a scheduled visit	Asks for a change in schedule or services

In a medical emergency, call 911 first and stay with the client. If the client has a valid Out-of-Hospital Do-Not-Resuscitate order (or identification device), show it to emergency responders; otherwise, staff who are trained in CPR should begin CPR while waiting for help. Complete an incident report for every fall, injury, medication error, exposure, or unusual event, and give it to your supervisor before the end of your shift. Incident reports are used to improve care and are not placed in the client record.



5.6 Infection Prevention

- **Hand hygiene** before and after every client contact, after removing gloves, and before handling clean supplies. Use alcohol-based hand rub unless hands are visibly soiled; then wash with soap and water. Your competency is checked at hire (Form E10).
- **Standard precautions** with every client: gloves for contact with blood, body fluids, mucous membranes, or broken skin; gown, mask, and eye protection when splashes are possible.
- **Bag technique:** place your bag on a clean, dry surface or barrier; take out what you need before care; never put soiled items back into the bag.
- **Sharps:** never recap, bend, or break needles; place them in a sharps container immediately.
- **Do not work sick.** If you have a fever, vomiting, diarrhea, or a contagious respiratory illness, call off and tell your supervisor.
- **Vaccines and TB:** the Agency offers the Hepatitis B vaccine at no cost to staff with exposure risk (Form E07), asks about influenza vaccination each season (Form E09), and screens for tuberculosis at hire and yearly (Form E08).



5.7 Blood and Body Fluid Exposure

If you are stuck by a used needle or get blood or body fluid in your eyes, mouth, or broken skin: wash the area with soap and water (flush eyes or mouth with water); call the office immediately; and go for medical evaluation the same day — some preventive treatment works best within hours. The Agency follows the OSHA Bloodborne Pathogens Standard (29 CFR 1910.1030), provides follow-up at no cost to you, and keeps exposure records confidential.



5.8 Safety in the Home

- Use good body mechanics: bend your knees, keep the load close, and do not twist. Use a gait belt and the equipment on the care plan for transfers. Ask for help or call the office instead of lifting more than you safely can.
- Look for fall hazards — rugs, cords, clutter, poor lighting — and report them so the nurse can address them (Home Safety Checklist, Form A09).
- Ask that pets be secured during visits. Park in well-lit areas, keep your car locked, and keep your phone charged.
- Follow oxygen safety rules: no smoking or open flames near oxygen.



5.9 Emergency Preparedness and Severe Weather

The Agency has a written emergency preparedness and response plan. Every client is assigned a disaster priority level (I through IV) at admission (Form A17); during a disaster, visits are scheduled by priority so that the most vulnerable clients are seen first. During an emergency or severe weather:

- keep your phone on and answer calls from the office; tell us whether you can safely work;
- do not drive into flooded roads or unsafe conditions — call the office for instructions;
- help clients follow their Family Emergency Preparedness and Response Plan (Form A10) and remind those who need evacuation help to register with STEAR by dialing 2-1-1.

You will receive emergency preparedness training during orientation and every year.

PART 6
Role Guides

Each guide lists what you need before you start, what you must keep current, the rules that matter most in your role, and where to find official information. These guides add to — they do not replace — Parts 2 through 5 and your job description.



6.1 Registered Nurses

Case managers, admitting nurses, clinical supervisors, and the Supervising Nurse

Before you start	Active Texas RN license or multistate license under the Nurse Licensure Compact, verified with the Texas Board of Nursing (BON). Current CPR (Basic Life Support) certification. Orientation, OASIS-E2 training, and EVV/clinical record training.
Keep current	License renewal every two years with the continuing education the BON requires (check bon.texas.gov for the topics that apply to you). CPR certification; annual in-service topics (Part 7).
Rules that matter most	Complete the initial assessment within 48 hours of referral (or on the ordered start-of-care date) and the comprehensive assessment with OASIS within 5 days after start of care; complete recertification, resumption-of-care, transfer, and discharge assessments in the required windows (42 CFR 484.55). Supervise home health aides on site at least every 14 days for clients receiving skilled services (one virtual visit allowed per 60-day episode) and every 60 days for aide-only clients (42 CFR 484.80). Delegate tasks to unlicensed staff only as allowed by BON Rule 225 (independent living environments) and document the delegation and training. Coordinate with the physician or allowed practitioner; obtain and document orders; update the plan of care.
Resources	Texas Board of Nursing: bon.texas.gov (license verification, Nurse Practice Act, rules, delegation resource packet) Medicare Home Health Conditions of Participation: ecfr.gov — 42 CFR Part 484 OASIS-E2 Guidance Manual: cms.gov (Home Health Quality Reporting) HHSC HCSSA rules: Texas Administrative Code, Title 26, Chapter 558 (sos.state.tx.us)



6.2 Licensed Vocational Nurses

Before you start	Active Texas LVN license or multistate license, verified with the BON. Current CPR (BLS) certification; orientation and documentation training.

Keep current	License renewal every two years with BON-required continuing education. CPR certification; annual in-service topics.
Rules that matter most	Work under the direction of a registered nurse and within the LVN scope of practice (BON Rule 217.11). LVNs perform focused assessments and provide care on the plan of care; comprehensive assessments and OASIS start-of-care, resumption, recertification, and discharge assessments are completed by an RN or qualified therapist. Report changes in condition to the case manager RN promptly.
Resources	Texas Board of Nursing: bon.texas.gov Medicare Home Health Conditions of Participation: ecfr.gov — 42 CFR Part 484



6.3 Therapists and Therapy Assistants

Physical therapy, occupational therapy, and speech-language pathology

Before you start	Active Texas license verified with your board: PT, PTA, OT, and COTA with the Texas Board of Physical Therapy and Occupational Therapy Examiners; SLP and SLP assistants with the Texas Department of Licensing and Regulation. Current CPR (BLS) certification; orientation and documentation training.
Keep current	License renewal and continuing education required by your board. Annual in-service topics.
Rules that matter most	A qualified therapist (not an assistant) must perform a functional reassessment at least once every 30 days and document measurable progress toward goals. Assistants work under the supervision required by their board's rules. For therapy-only cases, complete the comprehensive assessment and OASIS as Medicare rules allow. Communicate changes and coordinate with the case manager.
Resources	Texas Board of PT and OT Examiners: ptot.texas.gov Texas Department of Licensing and Regulation, Speech-Language Pathologists: tdlr.texas.gov Medicare coverage of therapy in home health: ecfr.gov — 42 CFR 409.44



6.4 Home Health Aides

Aides serving Medicare and other skilled home health clients

Before you start	Completion of an approved training program (at least 75 hours, including 16 hours of supervised practical training) and a competency evaluation, or listing in good standing on the Texas Nurse Aide Registry as allowed by Medicare rules. Registry and criminal history checks cleared; orientation; hand hygiene and skills competency. Current CPR certification before your first client visit.
Keep current	At least 12 hours of in-service training in every 12-month period (42 CFR 484.80). CPR certification (renew before your card expires). If you go 24 months in a row without paid aide work, you must retrain before working.
Rules that matter most	Follow the written aide care plan; do only the tasks on it. Your RN supervisor will visit to supervise your care; cooperate with these visits. Report changes in the client's condition to the RN the same day — right away if urgent. Do not give medications except as allowed under 5.3.
Resources	HHSC Nurse Aide Registry: hhs.texas.gov (search "Nurse Aide Registry") Medicare home health aide requirements: ecfr.gov — 42 CFR 484.80



6.5 Personal Care Attendants

Attendants providing personal assistance services (Medicaid, managed care, VA, private pay)

Before you start	At least 18 years old; registry, criminal history, and exclusion checks cleared. Agency orientation (Form E14) and a client-specific orientation by your supervisor before your first visit with each client. EVV training and an HHAeXchange profile. Current CPR certification before your first client visit.
Keep current	Annual in-service training on the Agency's required topics (Part 7). CPR certification (renew before your card expires). Annual registry re-check (done by the Agency).

Rules that matter most	Provide only the tasks on the client's individualized service plan, on the authorized days and hours. Clock in and out of every visit with EVV and complete the service record. Tasks that require nursing judgment may be done only if a registered nurse has delegated them to you in writing for that client. Never leave a dependent client alone if your relief does not arrive — call the office.
Resources	HHSC Electronic Visit Verification: hhs.texas.gov/providers (search "EVV") HHAeXchange: hhaexchange.com Texas Abuse Hotline: txabusehotline.org



6.6 Office, Intake, Scheduling, and Billing Staff

Before you start	Screening cleared; orientation; privacy training; Electronic Data and System Access Confidentiality Statement (Form E16) signed before system access.
Keep current	Annual privacy, compliance, and emergency preparedness training.
Rules that matter most	Answer and log every call; route clinical calls to the on-call nurse; log every client complaint and give it to the Administrator the same day. Release records only as the Notice of Privacy Practices and the Privacy Officer allow. Complete EVV visit maintenance promptly with accurate reason codes; never change visit times without documentation. Never submit a claim unless the visit is documented, signed, and matches EVV and the authorization. Run monthly exclusion checks and keep personnel files organized as shown in Appendix A.
Resources	HHSC EVV Policy Handbook: hhs.texas.gov (Handbooks) HHS-OIG exclusion list: exclusions.oig.hhs.gov Texas HHSC-OIG exclusion list: oig.hhs.texas.gov



6.7 Administrator, Alternate Administrator, and Supervising Nurse

Before you start	Meet HHSC qualifications for the position and complete the initial administrator training HHSC requires (26 TAC § 558.259).
Keep current	Continuing education in agency administration each year as required by 26 TAC § 558.260.

Rules that matter most	Keep the agency license, policies, emergency preparedness plan, and quality assessment and performance improvement (QAPI) program current. Make required reports to HHSC (for example, suspected abuse, neglect, or exploitation by staff) within HHSC timeframes, and cooperate with surveys and investigations. Ensure every employee is screened, oriented, trained, supervised, and evaluated, and that personnel files are complete (Appendix A).
Resources	HHSC Home and Community Support Services Agencies: hhs.texas.gov/providers Texas Administrative Code, Title 26, Chapter 558: sos.state.tx.us Medicare Home Health Conditions of Participation: ecfr.gov — 42 CFR Part 484

PART 7

Training and Competency



7.1 Orientation

Before you provide care, you complete orientation using the Employee Orientation Checklist (Form E13) and, for attendants, the Attendant Orientation Checklist (Form E14). Items marked with an asterisk on those checklists — including privacy, exposure control, client rights, and safety — must be complete before your first client contact. Privacy training must be documented within 90 days of hire (Form E19).



7.2 Annual In-Service Training

Every employee completes the Agency's required annual training. Home health aides must complete at least 12 hours of in-service every 12 months. Training is paid time, and records are kept in Section 3 of your personnel file.

Required every year (all staff)	Required every year (field staff)
Abuse, neglect, and exploitation — recognition and reporting	Infection prevention and bloodborne pathogens (OSHA)
Privacy and security of client information	Client rights and Rights of the Elderly
Compliance: fraud, waste, abuse, and the anti-kickback rules	Home safety, fall prevention, and body mechanics
Emergency preparedness, fire safety, and severe weather	EVV and documentation
Harassment prevention and workplace violence	Role-specific skills reviewed by your supervisor

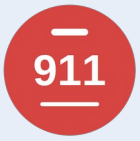


7.3 Competency and CPR

Your supervisor checks your skills before you perform them for clients and at least yearly, and whenever a new task is added to a care plan. Every employee who provides care in clients' homes — nurses, therapists, home health aides, and personal care attendants — must have current CPR certification before the first client visit and must keep it current. Nurses and therapists must hold Basic Life Support (BLS) certification. Give the office a copy of your card, and of each renewal card, before your current card expires. You may not work with clients while your CPR card is expired.

PART 8
Resource Directory

Official sources change from time to time. If a link no longer works, search the agency name, or ask the office for help.



8.1 Emergencies and Reporting

Resource	Phone	Website / what it is for
Emergency	911	Police, fire, ambulance
Texas Abuse Hotline (DFPS)	1-800-252-5400	txabusehotline.org — report abuse, neglect, or exploitation (24 hours)
HHSC Complaint and Incident Intake	1-800-458-9858	hhs.texas.gov — complaints about agencies; ANE by agency staff
Poison Control	1-800-222-1222	poisonhelp.hrsa.gov
988 Suicide & Crisis Lifeline	988	988lifeline.org — call or text, 24 hours
State of Texas Emergency Assistance Registry	2-1-1	stear.tdem.texas.gov — evacuation help registration



8.2 Licensing Boards and Registries

Resource	Phone	Website / what it is for
Texas Board of Nursing	512-305-7400	bon.texas.gov — RN/LVN licenses, Nurse Practice Act, delegation rules
Texas Board of PT and OT Examiners	512-305-6900	ptot.texas.gov — PT, PTA, OT, COTA licenses
Texas Department of Licensing and Regulation	1-800-803-9202	tdlr.texas.gov — speech-language pathologists
HHSC Nurse Aide Registry and Employee Misconduct Registry		hhs.texas.gov — searched through HHSC TULIP
Texas Department of Public Safety		dps.texas.gov — criminal history checks



8.3 Rules and Regulations

Resource	Phone	Website / what it is for
Medicare Home Health Conditions of Participation		ecfr.gov — 42 CFR Part 484

Texas HCSSA licensing rules		sos.state.tx.us — Texas Administrative Code, Title 26, Chapter 558
Texas statutes (Health and Safety Code Ch. 142, 181, 250; Human Resources Code Ch. 48, 102)		statutes.capitol.texas.gov
HHSC Electronic Visit Verification		hhs.texas.gov — Providers > Long-Term Care Provider Resources > EVV
OASIS-E2 Guidance Manual		cms.gov — Home Health Quality Reporting
OSHA Bloodborne Pathogens Standard	1-800-321-6742	osha.gov — 29 CFR 1910.1030
CDC Hand Hygiene and Infection Control		cdc.gov



8.4 Your Rights as an Employee

Resource	Phone	Website / what it is for
U.S. Department of Labor, Wage and Hour Division	1-866-487-9243	dol.gov/agencies/whd — minimum wage and overtime
Texas Workforce Commission	1-800-939-6631	twc.texas.gov — Payday Law wage claims, unemployment, Civil Rights Division
U.S. Equal Employment Opportunity Commission	1-800-669-4000	eeoc.gov — discrimination and harassment
Texas Department of Insurance, Division of Workers' Compensation	1-800-252-7031	tdi.texas.gov/wc
HHS Office of Inspector General Hotline	1-800-447-8477	tips.hhs.gov — report Medicare or Medicaid fraud
Texas HHSC Office of Inspector General	1-800-436-6184	oig.hhs.texas.gov — report Medicaid fraud

APPENDIX A

Where Your Signed Documents Live

This map shows where each form you sign is filed. Managers use it to set up and audit personnel files. Each personnel file has five tabbed sections: (1) Hiring and Screening; (2) Policies and Acknowledgments; (3) Training and Competency; (4) Performance and Discipline; (5) Payroll and Contact Information. Medical, Form I-9, and criminal history records are kept in separate locked files, never in the personnel file.

Form	Document	When	Where it is filed	Kept at least
E00	New-Hire Packet and Personnel File Checklist	At hire	Personnel file, front	5 years after employment ends*
E01–E04	Application; references; background check disclosure; Statement of Employability	Before hire	Personnel file, Section 1	5 years after employment ends*
—	Registry (NAR/EMR), exclusion (LEIE/HHSC-OIG), and license verification printouts	Before hire; registries yearly; exclusions monthly	Personnel file, Section 1	5 years after employment ends*
—	DPS CCH Verification Form; Criminal History Review Memo (E20)	Before hire	Criminal history file (separate, locked)	Per DPS rules; criminal history records are not stored
E21	CCH Name-Based Search Tracking Log	Each search	Agency compliance binder	Per DPS audit rules
E05, E06, E15, E16, E18, E23, E24	Policy acknowledgments; drug-free policy; confidentiality / conflict of interest; system access; EMR notice; handbook receipt; notice of no workers' compensation coverage	At hire (E18 within 5 working days)	Personnel file, Section 2	5 years after employment ends*
E11 / E12	Signed job description and evaluation	At hire; yearly	Personnel file, Sections 2 and 4	5 years after employment ends*
E10, E13, E14, E19	Hand hygiene competency; orientation checklists; privacy training statement	Before client contact; E19 within 90 days	Personnel file, Section 3	E19: 6 years from signing; others 5 years*
—	In-service records, skills checklists, CPR cards, license copies	Ongoing	Personnel file, Section 3	5 years after employment ends*
—	Evaluations, warnings, commendations	Ongoing	Personnel file, Section 4	5 years after employment ends*
—	W-4, direct deposit, emergency contact, driver's license and auto insurance	At hire; on change	Personnel file, Section 5	At least 4 years (tax records)
E07, E08, E09	Hepatitis B, tuberculosis, and influenza forms; exposure and injury records; accommodation records	At hire; yearly; as needed	Confidential medical file	Hepatitis B and exposure records: length of employment plus 30 years (OSHA)
E22	Employee Influenza Vaccination Log	Each flu season	Agency infection control binder	5 years*
I-9	Form I-9 and document copies	Day 1 and within 3 business days	Form I-9 file (separate)	3 years after hire or 1 year after employment ends, whichever is later
—	Timesheets, EVV records, payroll records	Each pay period	Payroll / EVV system	At least 3 years (FLSA)

** Agency retention policy. A longer period applies if a law, payer contract, investigation, or lawsuit requires it.*

APPENDIX B

ACKNOWLEDGMENT OF RECEIPT OF EMPLOYEE HANDBOOK

I acknowledge that I received the One Community Home Health Employee Handbook and Policy Manual (effective October 1, 2026), in paper or electronic form, and that I am responsible for reading it. I understand that:

- I may ask my supervisor or the Administrator any questions about the handbook, and I know where to find the Agency's Policy and Procedure Manual and the full text of the policies I sign.
- I must follow the policies in the handbook, including the policies on confidentiality, the drug-free workplace, harassment, abuse and neglect reporting, electronic visit verification, and professional boundaries.
- The handbook is not a contract of employment. My employment is at will, and only the Administrator can change that in a signed written agreement.
- The Agency may change its policies, and I will be told about material changes.
- The handbook (Appendix A) explains where my signed documents are filed, and I may review my personnel file by appointment.

Employee name (print): _____ Position: _____

_____ _____

Employee signature *Date*

_____ _____

Agency representative *Date*

Original filed in Section 2 of the employee's personnel file. Employee may request a copy.